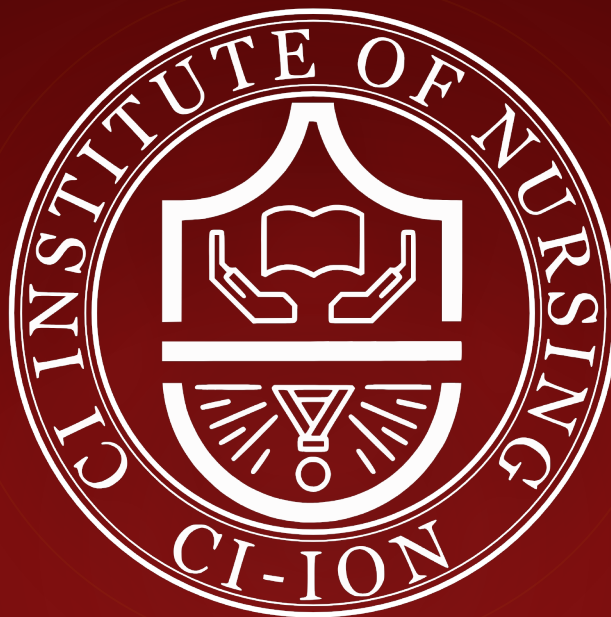




CALIFORNIA BRN COURSE MATERIALS

30 CONTACT HOUR PACKET

ONLINE CE – SIGNER-REVIEW BINDER

30 Contact Hours | 10 Modules × 3 Contact Hours | RN Care Manager CEP

CONTACT-HOUR STRUCTURE REMAINS DRAFT / PENDING BRN CEP APPROVAL METADATA

CI Institute of Nursing, Inc.

STATUS: DRAFT / PENDING BRN CEP APPROVAL

Ready for verification, signature, and submission review.

BRN RN CASE MANAGEMENT 30 CONTACT-HOUR MOODLE COURSE CONTENT

Status: Draft / Pending BRN CEP Approval

Course title: RN Case Management in Home Health: Care Coordination, Documentation, Patient Advocacy, and Safe Transitions of Care

Total target: 30 contact hours.

Structure: 10 modules x 3 contact hours each.

Approval guardrail: This content is prepared for a pending BRN Continuing Education Provider lane. It must not be described as BRN-approved, and no certificate may be issued with BRN approval language until provider approval and required metadata are available.

Terminology guardrail: Use contact hours as the primary measurement. This package is measured in contact hours, not CEUs.

Course-Level Learning Outcomes

By the end of the course, the learner will be able to:

- Apply RN case management principles to assessment, care planning, coordination, patient advocacy, documentation, transitions, utilization communication, quality escalation, and high-risk patient management.
- Select patient-centered coordination actions that remain within RN scope and support direct or indirect patient/client care.
- Produce audit-ready documentation that shows assessment findings, nursing actions, communication, follow-up, patient response, and unresolved barriers.
- Complete case-based Moodle interactions, documentation exercises, quizzes, reflections, and a final capstone assessment.
- Follow completion and certificate guardrails while the course remains draft pending BRN CEP approval.

Moodle Shell Pattern

Each module should be built with the following Moodle activity blocks:

- Page: Module Overview
- Page: Lesson Content
- Lesson/H5P: Case Scenario
- Assignment: Documentation/Application Exercise
- Quiz: Knowledge Check
- Feedback/Attestation: Learner Reflection
- Completion Gate: Required completion

M01 – FOUNDATIONS OF RN CASE MANAGEMENT AND NURSING SCOPE

Page: Module Overview

Module ID: M01

Module title: Foundations of RN Case Management and Nursing Scope

Contact-hour target: 3 contact hours.

Designed Moodle time support: approximately 170 learner minutes across lesson reading, case scenario decisions, active interaction, documentation exercise, knowledge check quiz, and reflection. The contact-hour target remains 3 contact hours.

Module purpose: RN case management as a nursing process that integrates assessment, planning, coordination, communication, evaluation, and advocacy within the registered nurse scope of practice.

Learning objectives:

- Differentiate distinguish RN case management from general administrative case handling in an RN case management context.
- Apply connect indirect patient care to nursing assessment and clinical judgment to realistic direct or indirect patient-care decisions.
- Document apply scope-of-practice boundaries during coordination and referral decisions using factual, audit-ready nursing language.
- Select appropriate coordination, communication, and escalation steps for use patient-centered goals to organize the case management plan.
- Evaluate learner decisions against scope, patient safety, and Draft / Pending BRN CEP Approval compliance guardrails.

Page: Lesson Content

Lesson Segment 1: Distinguish Rn Case Management From General Administrative Case Handling

In this segment, the learner studies how to distinguish RN case management from general administrative case handling while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Foundations of RN Case Management and Nursing Scope, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

BRN 30 CONTACT-HOUR COURSE MATERIALS

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When connect indirect patient care to nursing assessment and clinical judgment is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 2: Connect Indirect Patient Care To Nursing Assessment And Clinical Judgment

In this segment, the learner studies how to connect indirect patient care to nursing assessment and clinical judgment while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Foundations of RN Case Management and Nursing Scope, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When apply scope-of-practice boundaries during coordination and referral decisions is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 3: Apply Scope-Of-Practice Boundaries During Coordination And Referral Decisions

In this segment, the learner studies how to apply scope-of-practice boundaries during coordination and referral decisions while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Foundations of RN Case Management and Nursing Scope, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When use patient-centered goals to organize the case management plan is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 4: Use Patient-Centered Goals To Organize The Case Management Plan

BRN 30 CONTACT-HOUR COURSE MATERIALS

In this segment, the learner studies how to use patient-centered goals to organize the case management plan while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Foundations of RN Case Management and Nursing Scope, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When recognize when a case requires escalation to a provider, supervisor, or interdisciplinary partner is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 5: Recognize When A Case Requires Escalation To A Provider, Supervisor, Or Interdisciplinary Partner

In this segment, the learner studies how to recognize when a case requires escalation to a provider, supervisor, or interdisciplinary partner while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Foundations of RN Case Management and Nursing Scope, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When document case management actions as nursing care coordination rather than sales or business activity is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Practice Integration

The module closes by asking the learner to combine the lesson content with the scenario activity and documentation exercise. Learners should be able to explain how their decisions protect patient safety, respect patient rights, support continuity of care, and preserve scope boundaries. The activity is designed for Moodle delivery through a page lesson, case scenario, assignment, quiz, and attestation. Together these activities reasonably support the 3 contact-hour target through reading, applied decision-making, documentation practice, knowledge checks, and reflection.

Lesson/H5P: Case Scenario

An RN case manager receives a new home health referral for an older adult recently discharged after heart failure exacerbation. The referral packet includes medication changes, a low-sodium diet order, a pending cardiology appointment, and a family caregiver who is unsure which symptoms should trigger a call. The RN must decide what belongs in the nursing case management plan, what should be escalated, and what should not be presented as independent medical diagnosis.

BRN 30 CONTACT-HOUR COURSE MATERIALS

Learner task: Review the case, identify the highest-priority patient-care risks, choose the RN case management actions that fit the module objectives, and explain what must be documented or escalated. The Moodle builder may configure this as an H5P branching scenario, Moodle Lesson, or case-response activity.

Expected case output:

- One priority patient-care risk.
- One patient-centered goal.
- Two RN case management actions.
- One communication or escalation step.
- One documentation element that must appear in the record.

Active Learner Interaction

Scope sorting activity: learners drag actions into RN case management, provider-only decision, administrative support, or escalation-needed categories, then explain two borderline choices.

Moodle build note: Require learner submission or completion tracking. The interaction should require the learner to make choices, not only read text.

Assignment: Documentation/Application Exercise

Draft a one-page case management scope note that states the patient goal, RN coordination actions, escalation triggers, and what information remains outside the RN's independent authority.

Submission format: Moodle online text, file upload, or structured worksheet. Require original learner work and retain timestamped submission evidence.

Rubric criteria:

- Patient-care relevance and RN scope alignment.
- Clear priority setting.
- Factual, complete, and audit-ready documentation.
- Appropriate communication and escalation.
- Compliance with pending-approval and contact-hour terminology guardrails.

Quiz: Knowledge Check

Configure the Moodle quiz from the matching CSV rows in 03_BRN_QUESTION_BANK_MOODLE_IMPORT_READY.csv. Minimum quiz size for this module: 15 multiple-choice questions. Recommended passing score: 80 percent, with retake allowed according to provider policy.

Knowledge checks with answers and rationales:

BRN 30 CONTACT-HOUR COURSE MATERIALS

- M01 - Foundations of RN Case Management and Nursing Scope: Which answer best reflects RN scope boundaries for an RN case manager in this course?
- Answer: A - Coordinates care using nursing assessment findings and does not independently diagnose new medical conditions
- Rationale: Answer A is correct because coordinates care using nursing assessment findings and does not independently diagnose new medical conditions. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M01 - Foundations of RN Case Management and Nursing Scope: Which answer best reflects indirect patient care for an RN case manager in this course?
- Answer: A - Can include referral follow-up, education coordination, and communication that supports safe patient outcomes
- Rationale: Answer A is correct because can include referral follow-up, education coordination, and communication that supports safe patient outcomes. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M01 - Foundations of RN Case Management and Nursing Scope: Which answer best reflects patient-centered planning for an RN case manager in this course?
- Answer: A - Starts with the patient's clinical risks, preferences, functional needs, and measurable goals
- Rationale: Answer A is correct because starts with the patient's clinical risks, preferences, functional needs, and measurable goals. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M01 - Foundations of RN Case Management and Nursing Scope: Which answer best reflects escalation for an RN case manager in this course?
- Answer: A - Is required when assessment findings suggest a change in condition or an unresolved safety risk
- Rationale: Answer A is correct because is required when assessment findings suggest a change in condition or an unresolved safety risk. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M01 - Foundations of RN Case Management and Nursing Scope: Which answer best reflects role clarity for an RN case manager in this course?
- Answer: A - Prevents the RN from substituting payer, vendor, or business priorities for patient care needs
- Rationale: Answer A is correct because prevents the RN from substituting payer, vendor, or business priorities for patient care needs. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.

Moodle quiz questions for this module:

BRN 30 CONTACT-HOUR COURSE MATERIALS

QUESTION NAME	TOPIC AREA	DIFFICULTY
M01_Q01_RN_SCOPE_BOUNDARIES	RN scope boundaries	foundational
M01_Q02_INDIRECT_PATIENT_CARE	indirect patient care	foundational
M01_Q03_PATIENT_CENTERED_PLANNING	patient-centered planning	foundational
M01_Q04_ESCALATION	escalation	foundational
M01_Q05_ROLE_CLARITY	role clarity	foundational
M01_Q06_CASE_MANAGEMENT_PLAN	case management plan	applied
M01_Q07_INTERDISCIPLINARY_PRACTICE	interdisciplinary practice	applied
M01_Q08_PROFESSIONAL_ACCOUNTABILITY	professional accountability	applied
M01_Q09_ADVOCACY	advocacy	applied
M01_Q10_CARE_COORDINATION_EVIDENCE	care coordination evidence	applied
M01_Q11_ADULT_LEARNING_ACTIVITY	adult learning activity	applied
M01_Q12_APPROVAL_STATUS	approval status	advanced
M01_Q13_CONTACT_HOUR_LANGUAGE	contact-hour language	advanced
M01_Q14_RN_JUDGMENT	RN judgment	advanced
M01_Q15_COMPLETION_EVIDENCE	completion evidence	advanced

Feedback/Attestation: Learner Reflection

Prompt: Describe one decision from this module that changed how you would coordinate care for a patient. Identify the patient-care risk, the RN case management action, and the documentation or escalation step that supports safe follow-up.

Attestation statement: I attest that I completed the required module activities myself, reviewed the learner content, participated in the active interaction, completed the documentation/application exercise, and answered the knowledge check questions.

Completion Gate: Required completion

Completion rule: Mark the module complete only after the learner views the overview and lesson pages, completes the scenario activity, submits the assignment, passes the knowledge check quiz, and submits the reflection/attestation.

Evidence Generated By Moodle

- Page completion logs.
- Lesson/H5P or interaction completion report.
- Assignment submission timestamp and rubric grade.
- Quiz attempt, score, item responses, and pass status.
- Reflection/attestation timestamp.
- Completion gate status.

Instructor/Admin Notes

Review documentation exercises for RN scope, patient-care relevance, factual language, and escalation accuracy. Do not insert provider approval language or certificate release settings until BRN CEP approval metadata is available. If instructor-specific feedback is provided, retain it with course records according to provider policy.

Mo2 - ASSESSMENT, INTAKE, AND PATIENT-CENTERED CARE PLANNING

Compliance Guardrails

- Keep all build and learner-facing status as Draft / Pending BRN CEP Approval until approval is issued.
- Use contact hours as the primary measurement.
- Do not represent this content as BRN-approved before provider approval is issued.
- Do not copy official BRN forms, government form text, signer packets, or approval signatures into this Moodle content folder.
- Do not issue a certificate unless all completion, assessment, attestation, and provider-metadata rules are satisfied.

Page: Module Overview

Module ID: M02

Module title: Assessment, Intake, and Patient-Centered Care Planning

Contact-hour target: 3 contact hours.

Designed Moodle time support: approximately 170 learner minutes across lesson reading, case scenario decisions, active interaction, documentation exercise, knowledge check quiz, and reflection. The contact-hour target remains 3 contact hours.

Module purpose: Structured intake, risk identification, social and functional assessment, and conversion of assessment findings into a patient-centered RN case management plan.

Learning objectives:

- Differentiate collect intake data from referral, patient interview, caregiver report, and available records in an RN case management context.
- Apply identify priority clinical, functional, medication, psychosocial, and environmental risks to realistic direct or indirect patient-care decisions.
- Document write measurable care coordination goals that reflect patient preferences using factual, audit-ready nursing language.
- Select appropriate coordination, communication, and escalation steps for recognize gaps in information that require follow-up before the plan is complete.
- Evaluate learner decisions against scope, patient safety, and Draft / Pending BRN CEP Approval compliance guardrails.

Page: Lesson Content

Lesson Segment 1: Collect Intake Data From Referral, Patient Interview, Caregiver Report, And Available Records

BRN 30 CONTACT-HOUR COURSE MATERIALS

In this segment, the learner studies how to collect intake data from referral, patient interview, caregiver report, and available records while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Assessment, Intake, and Patient-Centered Care Planning, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When identify priority clinical, functional, medication, psychosocial, and environmental risks is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 2: Identify Priority Clinical, Functional, Medication, Psychosocial, And Environmental Risks

In this segment, the learner studies how to identify priority clinical, functional, medication, psychosocial, and environmental risks while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Assessment, Intake, and Patient-Centered Care Planning, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When write measurable care coordination goals that reflect patient preferences is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 3: Write Measurable Care Coordination Goals That Reflect Patient Preferences

In this segment, the learner studies how to write measurable care coordination goals that reflect patient preferences while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Assessment, Intake, and Patient-Centered Care Planning, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When recognize gaps in information that require follow-up before the plan is complete is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

BRN 30 CONTACT-HOUR COURSE MATERIALS

Lesson Segment 4: Recognize Gaps In Information That Require Follow-Up Before The Plan Is Complete

In this segment, the learner studies how to recognize gaps in information that require follow-up before the plan is complete while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Assessment, Intake, and Patient-Centered Care Planning, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When integrate cultural, language, health literacy, and access needs into planning is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 5: Integrate Cultural, Language, Health Literacy, And Access Needs Into Planning

In this segment, the learner studies how to integrate cultural, language, health literacy, and access needs into planning while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Assessment, Intake, and Patient-Centered Care Planning, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When separate objective assessment findings from assumptions or unsupported conclusions is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Practice Integration

The module closes by asking the learner to combine the lesson content with the scenario activity and documentation exercise. Learners should be able to explain how their decisions protect patient safety, respect patient rights, support continuity of care, and preserve scope boundaries. The activity is designed for Moodle delivery through a page lesson, case scenario, assignment, quiz, and attestation. Together these activities reasonably support the 3 contact-hour target through reading, applied decision-making, documentation practice, knowledge checks, and reflection.

Lesson/H5P: Case Scenario

BRN 30 CONTACT-HOUR COURSE MATERIALS

A patient with diabetes and a new wound-care order begins services after a hospitalization. The intake packet lists insulin changes, transportation barriers, food insecurity, and a caregiver who works nights. The RN case manager must create a plan that addresses immediate safety, patient goals, and the practical barriers that could prevent adherence.

Learner task: Review the case, identify the highest-priority patient-care risks, choose the RN case management actions that fit the module objectives, and explain what must be documented or escalated. The Moodle builder may configure this as an H5P branching scenario, Moodle Lesson, or case-response activity.

Expected case output:

- One priority patient-care risk.
- One patient-centered goal.
- Two RN case management actions.
- One communication or escalation step.
- One documentation element that must appear in the record.

Active Learner Interaction

Intake priority builder: learners rank ten intake findings by urgency, then map the top five to RN case management actions.

Moodle build note: Require learner submission or completion tracking. The interaction should require the learner to make choices, not only read text.

Assignment: Documentation/Application Exercise

Complete a structured intake summary with objective findings, risk flags, missing information, patient-stated goals, and a draft care coordination plan.

Submission format: Moodle online text, file upload, or structured worksheet. Require original learner work and retain timestamped submission evidence.

Rubric criteria:

- Patient-care relevance and RN scope alignment.
- Clear priority setting.
- Factual, complete, and audit-ready documentation.
- Appropriate communication and escalation.
- Compliance with pending-approval and contact-hour terminology guardrails.

Quiz: Knowledge Check

Configure the Moodle quiz from the matching CSV rows in 03_BRN_QUESTION_BANK_MOODLE_IMPORT_READY.csv. Minimum quiz size for this module: 15 multiple-choice questions. Recommended passing score: 80 percent, with retake allowed according to provider policy.

Knowledge checks with answers and rationales:

BRN 30 CONTACT-HOUR COURSE MATERIALS

- M02 - Assessment, Intake, and Patient-Centered Care Planning: Which answer best reflects intake source validation for an RN case manager in this course?
- Answer: A - Compares referral data with patient, caregiver, medication, and record information
- Rationale: Answer A is correct because compares referral data with patient, caregiver, medication, and record information. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M02 - Assessment, Intake, and Patient-Centered Care Planning: Which answer best reflects risk prioritization for an RN case manager in this course?
- Answer: A - Places immediate safety and change-in-condition concerns ahead of routine preferences
- Rationale: Answer A is correct because places immediate safety and change-in-condition concerns ahead of routine preferences. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M02 - Assessment, Intake, and Patient-Centered Care Planning: Which answer best reflects patient-centered goals for an RN case manager in this course?
- Answer: A - Include what matters to the patient and are written in measurable language
- Rationale: Answer A is correct because include what matters to the patient and are written in measurable language. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M02 - Assessment, Intake, and Patient-Centered Care Planning: Which answer best reflects social determinants for an RN case manager in this course?
- Answer: A - Are included when they affect access, adherence, safety, or care transitions
- Rationale: Answer A is correct because are included when they affect access, adherence, safety, or care transitions. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M02 - Assessment, Intake, and Patient-Centered Care Planning: Which answer best reflects health literacy for an RN case manager in this course?
- Answer: A - Requires communication that the patient can understand and use
- Rationale: Answer A is correct because requires communication that the patient can understand and use. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.

Moodle quiz questions for this module:

BRN 30 CONTACT-HOUR COURSE MATERIALS

QUESTION NAME	TOPIC AREA	DIFFICULTY
M02_Q01_INTAKE_SOURCE_VALIDATION	intake source validation	foundational
M02_Q02_RISK_PRIORITIZATION	risk prioritization	foundational
M02_Q03_PATIENT_CENTERED_GOALS	patient-centered goals	foundational
M02_Q04_SOCIAL_DETERMINANTS	social determinants	foundational
M02_Q05_HEALTH_LITERACY	health literacy	foundational
M02_Q06_OBJECTIVE_DOCUMENTATION	objective documentation	applied
M02_Q07_MISSING_INFORMATION	missing information	applied
M02_Q08_MEDICATION_RISK	medication risk	applied
M02_Q09_FUNCTIONAL_ASSESSMENT	functional assessment	applied
M02_Q10_CULTURAL_RESPONSIVENESS	cultural responsiveness	applied
M02_Q11_CARE_PLAN_ALIGNMENT	care plan alignment	applied
M02_Q12_HANDOFF_READINESS	handoff readiness	advanced
M02_Q13_CONTACT_HOUR_BASIS	contact-hour basis	advanced
M02_Q14_LEARNER_ATTESTATION	learner attestation	advanced
M02_Q15_BRN_STATUS_LANGUAGE	BRN status language	advanced

Feedback/Attestation: Learner Reflection

Prompt: Describe one decision from this module that changed how you would coordinate care for a patient. Identify the patient-care risk, the RN case management action, and the documentation or escalation step that supports safe follow-up.

Attestation statement: I attest that I completed the required module activities myself, reviewed the learner content, participated in the active interaction, completed the documentation/application exercise, and answered the knowledge check questions.

Completion Gate: Required completion

Completion rule: Mark the module complete only after the learner views the overview and lesson pages, completes the scenario activity, submits the assignment, passes the knowledge check quiz, and submits the reflection/attestation.

Evidence Generated By Moodle

- Page completion logs.
- Lesson/H5P or interaction completion report.
- Assignment submission timestamp and rubric grade.
- Quiz attempt, score, item responses, and pass status.
- Reflection/attestation timestamp.
- Completion gate status.

Instructor/Admin Notes

Review documentation exercises for RN scope, patient-care relevance, factual language, and escalation accuracy. Do not insert provider approval language or certificate release settings until BRN CEP approval metadata is available. If instructor-specific feedback is provided, retain it with course records according to provider policy.

Mo3 - CARE COORDINATION ACROSS DISCIPLINES AND SETTINGS

Compliance Guardrails

- Keep all build and learner-facing status as Draft / Pending BRN CEP Approval until approval is issued.
- Use contact hours as the primary measurement.
- Do not represent this content as BRN-approved before provider approval is issued.
- Do not copy official BRN forms, government form text, signer packets, or approval signatures into this Moodle content folder.
- Do not issue a certificate unless all completion, assessment, attestation, and provider-metadata rules are satisfied.

Page: Module Overview

Module ID: M03

Module title: Care Coordination Across Disciplines and Settings

Contact-hour target: 3 contact hours.

Designed Moodle time support: approximately 170 learner minutes across lesson reading, case scenario decisions, active interaction, documentation exercise, knowledge check quiz, and reflection. The contact-hour target remains 3 contact hours.

Module purpose: Interdisciplinary communication, handoffs, referral management, team accountability, and closed-loop follow-up across care settings.

Learning objectives:

- Differentiate identify disciplines and partners needed for safe coordination in an RN case management context.
- Apply use closed-loop communication to prevent unresolved referrals and handoffs to realistic direct or indirect patient-care decisions.
- Document prepare concise clinical updates for providers and team members using factual, audit-ready nursing language.
- Select appropriate coordination, communication, and escalation steps for coordinate across home health, clinic, hospital, pharmacy, and community settings.
- Evaluate learner decisions against scope, patient safety, and Draft / Pending BRN CEP Approval compliance guardrails.

Page: Lesson Content

Lesson Segment 1: Identify Disciplines And Partners Needed For Safe Coordination

BRN 30 CONTACT-HOUR COURSE MATERIALS

In this segment, the learner studies how to identify disciplines and partners needed for safe coordination while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Care Coordination Across Disciplines and Settings, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When use closed-loop communication to prevent unresolved referrals and handoffs is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 2: Use Closed-Loop Communication To Prevent Unresolved Referrals And Handoffs

In this segment, the learner studies how to use closed-loop communication to prevent unresolved referrals and handoffs while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Care Coordination Across Disciplines and Settings, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When prepare concise clinical updates for providers and team members is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 3: Prepare Concise Clinical Updates For Providers And Team Members

In this segment, the learner studies how to prepare concise clinical updates for providers and team members while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Care Coordination Across Disciplines and Settings, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When coordinate across home health, clinic, hospital, pharmacy, and community settings is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

BRN 30 CONTACT-HOUR COURSE MATERIALS

Lesson Segment 4: Coordinate Across Home Health, Clinic, Hospital, Pharmacy, And Community Settings

In this segment, the learner studies how to coordinate across home health, clinic, hospital, pharmacy, and community settings while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Care Coordination Across Disciplines and Settings, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When track barriers until they are resolved, escalated, or reassigned is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 5: Track Barriers Until They Are Resolved, Escalated, Or Reassigned

In this segment, the learner studies how to track barriers until they are resolved, escalated, or reassigned while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Care Coordination Across Disciplines and Settings, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When document communication so the record shows what was requested, by whom, and with what result is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Practice Integration

The module closes by asking the learner to combine the lesson content with the scenario activity and documentation exercise. Learners should be able to explain how their decisions protect patient safety, respect patient rights, support continuity of care, and preserve scope boundaries. The activity is designed for Moodle delivery through a page lesson, case scenario, assignment, quiz, and attestation. Together these activities reasonably support the 3 contact-hour target through reading, applied decision-making, documentation practice, knowledge checks, and reflection.

Lesson/H5P: Case Scenario

BRN 30 CONTACT-HOUR COURSE MATERIALS

A home health patient has a therapy evaluation pending, a wound clinic appointment scheduled, and a pharmacy question about a high-risk medication. The patient also reports difficulty reaching the primary care office. The RN case manager must coordinate the team without letting any handoff remain open-ended.

Learner task: Review the case, identify the highest-priority patient-care risks, choose the RN case management actions that fit the module objectives, and explain what must be documented or escalated. The Moodle builder may configure this as an H5P branching scenario, Moodle Lesson, or case-response activity.

Expected case output:

- One priority patient-care risk.
- One patient-centered goal.
- Two RN case management actions.
- One communication or escalation step.
- One documentation element that must appear in the record.

Active Learner Interaction

Closed-loop communication simulation: learners choose what to send, whom to contact, and how to confirm receipt for four coordination problems.

Moodle build note: Require learner submission or completion tracking. The interaction should require the learner to make choices, not only read text.

Assignment: Documentation/Application Exercise

Create an interdisciplinary communication log that includes issue, recipient, urgency, message content, expected response, follow-up date, and outcome.

Submission format: Moodle online text, file upload, or structured worksheet. Require original learner work and retain timestamped submission evidence.

Rubric criteria:

- Patient-care relevance and RN scope alignment.
- Clear priority setting.
- Factual, complete, and audit-ready documentation.
- Appropriate communication and escalation.
- Compliance with pending-approval and contact-hour terminology guardrails.

Quiz: Knowledge Check

Configure the Moodle quiz from the matching CSV rows in 03_BRN_QUESTION_BANK_MOODLE_IMPORT_READY.csv. Minimum quiz size for this module: 15 multiple-choice questions. Recommended passing score: 80 percent, with retake allowed according to provider policy.

Knowledge checks with answers and rationales:

BRN 30 CONTACT-HOUR COURSE MATERIALS

- M03 - Care Coordination Across Disciplines and Settings: Which answer best reflects closed-loop communication for an RN case manager in this course?
- Answer: A - Requires confirmation that the message was received, understood, and acted on
- Rationale: Answer A is correct because requires confirmation that the message was received, understood, and acted on. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M03 - Care Coordination Across Disciplines and Settings: Which answer best reflects discipline selection for an RN case manager in this course?
- Answer: A - Matches patient need to the appropriate professional or service
- Rationale: Answer A is correct because matches patient need to the appropriate professional or service. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M03 - Care Coordination Across Disciplines and Settings: Which answer best reflects handoff quality for an RN case manager in this course?
- Answer: A - Includes current status, risk, requested action, and time sensitivity
- Rationale: Answer A is correct because includes current status, risk, requested action, and time sensitivity. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M03 - Care Coordination Across Disciplines and Settings: Which answer best reflects referral tracking for an RN case manager in this course?
- Answer: A - Keeps barriers visible until resolved or escalated
- Rationale: Answer A is correct because keeps barriers visible until resolved or escalated. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M03 - Care Coordination Across Disciplines and Settings: Which answer best reflects provider communication for an RN case manager in this course?
- Answer: A - Uses concise clinical facts and a clear request
- Rationale: Answer A is correct because uses concise clinical facts and a clear request. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.

Moodle quiz questions for this module:

QUESTION NAME	TOPIC AREA	DIFFICULTY
M03_Q01_CLOSED_LOOP_COMMUNICATION	closed-loop communication	foundational
M03_Q02_DISCIPLINE_SELECTION	discipline selection	foundational
M03_Q03_HANDOFF_QUALITY	handoff quality	foundational
M03_Q04_REFERRAL_TRACKING	referral tracking	foundational
M03_Q05_PROVIDER_COMMUNICATION	provider communication	foundational
M03_Q06_PHARMACY_COORDINATION	pharmacy coordination	applied
M03_Q07_COMMUNITY_RESOURCE_LINKAGE	community resource linkage	applied
M03_Q08_TEAM_ACCOUNTABILITY	team accountability	applied
M03_Q09_COMMUNICATION_DOCUMENTATION	communication documentation	applied
M03_Q10_CARE_SETTING_TRANSITION	care setting transition	applied
M03_Q11_PATIENT_INCLUSION	patient inclusion	applied
M03_Q12_ESCALATION_PATHWAY	escalation pathway	advanced
M03_Q13_MOODLE_EVIDENCE	Moodle evidence	advanced
M03_Q14_PENDING_APPROVAL_GUARDRAIL	pending approval guardrail	advanced
M03_Q15_CONTACT_HOURS	contact hours	advanced

Mo4 - DOCUMENTATION, LEGAL CHARTING, AND AUDIT-READY RECORDS

Feedback/Attestation: Learner Reflection

Prompt: Describe one decision from this module that changed how you would coordinate care for a patient. Identify the patient-care risk, the RN case management action, and the documentation or escalation step that supports safe follow-up.

Attestation statement: I attest that I completed the required module activities myself, reviewed the learner content, participated in the active interaction, completed the documentation/application exercise, and answered the knowledge check questions.

Completion Gate: Required completion

Completion rule: Mark the module complete only after the learner views the overview and lesson pages, completes the scenario activity, submits the assignment, passes the knowledge check quiz, and submits the reflection/attestation.

Evidence Generated By Moodle

- Page completion logs.
- Lesson/H5P or interaction completion report.
- Assignment submission timestamp and rubric grade.
- Quiz attempt, score, item responses, and pass status.
- Reflection/attestation timestamp.
- Completion gate status.

Instructor/Admin Notes

Review documentation exercises for RN scope, patient-care relevance, factual language, and escalation accuracy. Do not insert provider approval language or certificate release settings until BRN CEP approval metadata is available. If instructor-specific feedback is provided, retain it with course records according to provider policy.

Compliance Guardrails

- Keep all build and learner-facing status as Draft / Pending BRN CEP Approval until approval is issued.
- Use contact hours as the primary measurement.
- Do not represent this content as BRN-approved before provider approval is issued.
- Do not copy official BRN forms, government form text, signer packets, or approval signatures into this Moodle content folder.
- Do not issue a certificate unless all completion, assessment, attestation, and provider-metadata rules are satisfied.

BRN 30 CONTACT-HOUR COURSE MATERIALS

Page: Module Overview

Module ID: M04

Module title: Documentation, Legal Charting, and Audit-Ready Records

Contact-hour target: 3 contact hours.

Designed Moodle time support: approximately 170 learner minutes across lesson reading, case scenario decisions, active interaction, documentation exercise, knowledge check quiz, and reflection. The contact-hour target remains 3 contact hours.

Module purpose: Nursing documentation principles, legal charting, audit readiness, correction practices, and records that demonstrate case management activity.

Learning objectives:

- Differentiate write timely, factual, complete, and clinically relevant case management notes in an RN case management context.
- Apply document patient education, communication, referrals, and follow-up outcomes to realistic direct or indirect patient-care decisions.
- Document avoid vague, judgmental, copied, or unsupported charting using factual, audit-ready nursing language.
- Select appropriate coordination, communication, and escalation steps for correct documentation errors according to policy without hiding the original record.
- Evaluate learner decisions against scope, patient safety, and Draft / Pending BRN CEP Approval compliance guardrails.

Page: Lesson Content

Lesson Segment 1: Write Timely, Factual, Complete, And Clinically Relevant Case Management Notes

In this segment, the learner studies how to write timely, factual, complete, and clinically relevant case management notes while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Documentation, Legal Charting, and Audit-Ready Records, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When document patient education, communication, referrals, and follow-up outcomes is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 2: Document Patient Education, Communication, Referrals, And Follow-Up Outcomes

BRN 30 CONTACT-HOUR COURSE MATERIALS

In this segment, the learner studies how to document patient education, communication, referrals, and follow-up outcomes while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Documentation, Legal Charting, and Audit-Ready Records, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When avoid vague, judgmental, copied, or unsupported charting is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 3: Avoid Vague, Judgmental, Copied, Or Unsupported Charting

In this segment, the learner studies how to avoid vague, judgmental, copied, or unsupported charting while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Documentation, Legal Charting, and Audit-Ready Records, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When correct documentation errors according to policy without hiding the original record is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 4: Correct Documentation Errors According To Policy Without Hiding The Original Record

In this segment, the learner studies how to correct documentation errors according to policy without hiding the original record while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Documentation, Legal Charting, and Audit-Ready Records, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When build audit-ready evidence of contact-hour completion and learner performance is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

BRN 30 CONTACT-HOUR COURSE MATERIALS

Lesson Segment 5: Build Audit-Ready Evidence Of Contact-Hour Completion And Learner Performance

In this segment, the learner studies how to build audit-ready evidence of contact-hour completion and learner performance while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Documentation, Legal Charting, and Audit-Ready Records, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When protect confidentiality and minimum necessary information during communication is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Practice Integration

The module closes by asking the learner to combine the lesson content with the scenario activity and documentation exercise. Learners should be able to explain how their decisions protect patient safety, respect patient rights, support continuity of care, and preserve scope boundaries. The activity is designed for Moodle delivery through a page lesson, case scenario, assignment, quiz, and attestation. Together these activities reasonably support the 3 contact-hour target through reading, applied decision-making, documentation practice, knowledge checks, and reflection.

Lesson/H5P: Case Scenario

A chart audit questions whether a patient received discharge-medication teaching and whether an urgent provider call was completed. Notes exist in several places, but one note says only 'called doctor, handled.' The RN case manager must identify what is missing and rewrite the documentation in an audit-ready style.

Learner task: Review the case, identify the highest-priority patient-care risks, choose the RN case management actions that fit the module objectives, and explain what must be documented or escalated. The Moodle builder may configure this as an H5P branching scenario, Moodle Lesson, or case-response activity.

Expected case output:

- One priority patient-care risk.
- One patient-centered goal.
- Two RN case management actions.
- One communication or escalation step.
- One documentation element that must appear in the record.

Active Learner Interaction

Charting repair lab: learners review short documentation examples and classify them as complete, incomplete, late, subjective, or requiring correction.

BRN 30 CONTACT-HOUR COURSE MATERIALS

Moodle build note: Require learner submission or completion tracking. The interaction should require the learner to make choices, not only read text.

Assignment: Documentation/Application Exercise

Rewrite a deficient case management note using objective language, nursing assessment data, communication details, patient response, follow-up plan, and correction notation where needed.

Submission format: Moodle online text, file upload, or structured worksheet. Require original learner work and retain timestamped submission evidence.

Rubric criteria:

- Patient-care relevance and RN scope alignment.
- Clear priority setting.
- Factual, complete, and audit-ready documentation.
- Appropriate communication and escalation.
- Compliance with pending-approval and contact-hour terminology guardrails.

Quiz: Knowledge Check

Configure the Moodle quiz from the matching CSV rows in 03_BRN_QUESTION_BANK_MOODLE_IMPORT_READY.csv. Minimum quiz size for this module: 15 multiple-choice questions. Recommended passing score: 80 percent, with retake allowed according to provider policy.

Knowledge checks with answers and rationales:

BRN 30 CONTACT-HOUR COURSE MATERIALS

- M04 - Documentation, Legal Charting, and Audit-Ready Records: Which answer best reflects factual charting for an RN case manager in this course?
- Answer: A - Uses objective data, patient statements, nursing actions, and outcomes
- Rationale: Answer A is correct because uses objective data, patient statements, nursing actions, and outcomes. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M04 - Documentation, Legal Charting, and Audit-Ready Records: Which answer best reflects legal documentation for an RN case manager in this course?
- Answer: A - Must be timely, accurate, complete, and attributable
- Rationale: Answer A is correct because must be timely, accurate, complete, and attributable. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M04 - Documentation, Legal Charting, and Audit-Ready Records: Which answer best reflects audit-ready record for an RN case manager in this course?
- Answer: A - Shows what occurred, why it mattered, and what follow-up is pending
- Rationale: Answer A is correct because shows what occurred, why it mattered, and what follow-up is pending. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M04 - Documentation, Legal Charting, and Audit-Ready Records: Which answer best reflects late entry for an RN case manager in this course?
- Answer: A - Is labeled according to policy and does not pretend to be contemporaneous
- Rationale: Answer A is correct because is labeled according to policy and does not pretend to be contemporaneous. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M04 - Documentation, Legal Charting, and Audit-Ready Records: Which answer best reflects error correction for an RN case manager in this course?
- Answer: A - Preserves the original record and follows organizational procedure
- Rationale: Answer A is correct because preserves the original record and follows organizational procedure. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.

Moodle quiz questions for this module:

QUESTION NAME	TOPIC AREA	DIFFICULTY
M04_Q01_FACTUAL_CHARTING	factual charting	foundational
M04_Q02_LEGAL_DOCUMENTATION	legal documentation	foundational
M04_Q03_AUDIT_READY_RECORD	audit-ready record	foundational
M04_Q04_LATE_ENTRY	late entry	foundational
M04_Q05_ERROR_CORRECTION	error correction	foundational
M04_Q06_COPY_FORWARD_RISK	copy-forward risk	applied
M04_Q07_CONFIDENTIALITY	confidentiality	applied
M04_Q08_EDUCATION_DOCUMENTATION	education documentation	applied
M04_Q09_PROVIDER_NOTIFICATION	provider notification	applied
M04_Q10_REFERRAL_EVIDENCE	referral evidence	applied
M04_Q11_RECORDKEEPING_RETENTION	recordkeeping retention	applied
M04_Q12_MOODLE_LOGS	Moodle logs	advanced
M04_Q13_CERTIFICATE_CONTROL	certificate control	advanced
M04_Q14_PENDING_BRN_STATUS	pending BRN status	advanced
M04_Q15_CONTACT_HOUR_RECORD	contact-hour record	advanced

Mo5 - PATIENT RIGHTS, CONSENT, ADVOCACY, AND ETHICAL BOUNDARIES

Feedback/Attestation: Learner Reflection

Prompt: Describe one decision from this module that changed how you would coordinate care for a patient. Identify the patient-care risk, the RN case management action, and the documentation or escalation step that supports safe follow-up.

Attestation statement: I attest that I completed the required module activities myself, reviewed the learner content, participated in the active interaction, completed the documentation/application exercise, and answered the knowledge check questions.

Completion Gate: Required completion

Completion rule: Mark the module complete only after the learner views the overview and lesson pages, completes the scenario activity, submits the assignment, passes the knowledge check quiz, and submits the reflection/attestation.

Evidence Generated By Moodle

- Page completion logs.
- Lesson/H5P or interaction completion report.
- Assignment submission timestamp and rubric grade.
- Quiz attempt, score, item responses, and pass status.
- Reflection/attestation timestamp.
- Completion gate status.

Instructor/Admin Notes

Review documentation exercises for RN scope, patient-care relevance, factual language, and escalation accuracy. Do not insert provider approval language or certificate release settings until BRN CEP approval metadata is available. If instructor-specific feedback is provided, retain it with course records according to provider policy.

Compliance Guardrails

- Keep all build and learner-facing status as Draft / Pending BRN CEP Approval until approval is issued.
- Use contact hours as the primary measurement.
- Do not represent this content as BRN-approved before provider approval is issued.
- Do not copy official BRN forms, government form text, signer packets, or approval signatures into this Moodle content folder.
- Do not issue a certificate unless all completion, assessment, attestation, and provider-metadata rules are satisfied.

BRN 30 CONTACT-HOUR COURSE MATERIALS

Page: Module Overview

Module ID: M05

Module title: Patient Rights, Consent, Advocacy, and Ethical Boundaries

Contact-hour target: 3 contact hours.

Designed Moodle time support: approximately 170 learner minutes across lesson reading, case scenario decisions, active interaction, documentation exercise, knowledge check quiz, and reflection. The contact-hour target remains 3 contact hours.

Module purpose: Patient rights, informed participation, consent-aware communication, advocacy, confidentiality, boundaries, and ethical decision-making in RN case management.

Learning objectives:

- Differentiate recognize patient rights during assessment, planning, education, and coordination in an RN case management context.
- Apply support informed participation without coercion or misleading approval claims to realistic direct or indirect patient-care decisions.
- Document advocate for patient needs while respecting autonomy and scope boundaries using factual, audit-ready nursing language.
- Select appropriate coordination, communication, and escalation steps for manage confidentiality, family involvement, and consent-aware information sharing.
- Evaluate learner decisions against scope, patient safety, and Draft / Pending BRN CEP Approval compliance guardrails.

Page: Lesson Content

Lesson Segment 1: Recognize Patient Rights During Assessment, Planning, Education, And Coordination

In this segment, the learner studies how to recognize patient rights during assessment, planning, education, and coordination while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Patient Rights, Consent, Advocacy, and Ethical Boundaries, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When support informed participation without coercion or misleading approval claims is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 2: Support Informed Participation Without Coercion Or Misleading Approval Claims

BRN 30 CONTACT-HOUR COURSE MATERIALS

In this segment, the learner studies how to support informed participation without coercion or misleading approval claims while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Patient Rights, Consent, Advocacy, and Ethical Boundaries, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When advocate for patient needs while respecting autonomy and scope boundaries is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 3: Advocate For Patient Needs While Respecting Autonomy And Scope Boundaries

In this segment, the learner studies how to advocate for patient needs while respecting autonomy and scope boundaries while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Patient Rights, Consent, Advocacy, and Ethical Boundaries, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When manage confidentiality, family involvement, and consent-aware information sharing is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 4: Manage Confidentiality, Family Involvement, And Consent-Aware Information Sharing

In this segment, the learner studies how to manage confidentiality, family involvement, and consent-aware information sharing while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Patient Rights, Consent, Advocacy, and Ethical Boundaries, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When identify conflicts of interest, role confusion, and boundary risks is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

BRN 30 CONTACT-HOUR COURSE MATERIALS

Lesson Segment 5: Identify Conflicts Of Interest, Role Confusion, And Boundary Risks

In this segment, the learner studies how to identify conflicts of interest, role confusion, and boundary risks while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Patient Rights, Consent, Advocacy, and Ethical Boundaries, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When respond to ethical concerns through documentation, escalation, and consultation is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Practice Integration

The module closes by asking the learner to combine the lesson content with the scenario activity and documentation exercise. Learners should be able to explain how their decisions protect patient safety, respect patient rights, support continuity of care, and preserve scope boundaries. The activity is designed for Moodle delivery through a page lesson, case scenario, assignment, quiz, and attestation. Together these activities reasonably support the 3 contact-hour target through reading, applied decision-making, documentation practice, knowledge checks, and reflection.

Lesson/H5P: Case Scenario

A family member asks the RN case manager to withhold information from a patient about a discharge plan because the family fears the patient will refuse services. The patient is alert, asks direct questions, and wants to decide who receives updates. The RN must protect rights, communicate respectfully, and escalate concerns if needed.

Learner task: Review the case, identify the highest-priority patient-care risks, choose the RN case management actions that fit the module objectives, and explain what must be documented or escalated. The Moodle builder may configure this as an H5P branching scenario, Moodle Lesson, or case-response activity.

Expected case output:

- One priority patient-care risk.
- One patient-centered goal.
- Two RN case management actions.
- One communication or escalation step.
- One documentation element that must appear in the record.

Active Learner Interaction

Ethics branching scenario: learners choose responses to family pressure, patient refusal, confidentiality requests, and resource limitations.

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Moodle build note: Require learner submission or completion tracking. The interaction should require the learner to make choices, not only read text.

Assignment: Documentation/Application Exercise

Write an advocacy and consent-aware communication note that documents the patient preference, information shared, boundaries maintained, and escalation plan.

Submission format: Moodle online text, file upload, or structured worksheet. Require original learner work and retain timestamped submission evidence.

Rubric criteria:

- Patient-care relevance and RN scope alignment.
- Clear priority setting.
- Factual, complete, and audit-ready documentation.
- Appropriate communication and escalation.
- Compliance with pending-approval and contact-hour terminology guardrails.

Quiz: Knowledge Check

Configure the Moodle quiz from the matching CSV rows in 03_BRN_QUESTION_BANK_MOODLE_IMPORT_READY.csv. Minimum quiz size for this module: 15 multiple-choice questions. Recommended passing score: 80 percent, with retake allowed according to provider policy.

Knowledge checks with answers and rationales:

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- M05 - Patient Rights, Consent, Advocacy, and Ethical Boundaries: Which answer best reflects patient autonomy for an RN case manager in this course?
- Answer: A - Requires respecting the capable patient's informed choices
- Rationale: Answer A is correct because requires respecting the capable patient's informed choices. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M05 - Patient Rights, Consent, Advocacy, and Ethical Boundaries: Which answer best reflects consent-aware sharing for an RN case manager in this course?
- Answer: A - Limits family updates according to patient permission and applicable policy
- Rationale: Answer A is correct because limits family updates according to patient permission and applicable policy. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M05 - Patient Rights, Consent, Advocacy, and Ethical Boundaries: Which answer best reflects advocacy for an RN case manager in this course?
- Answer: A - Raises barriers and safety concerns without overriding the patient unnecessarily
- Rationale: Answer A is correct because raises barriers and safety concerns without overriding the patient unnecessarily. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M05 - Patient Rights, Consent, Advocacy, and Ethical Boundaries: Which answer best reflects ethical boundaries for an RN case manager in this course?
- Answer: A - Prevent coercion, favoritism, and role confusion
- Rationale: Answer A is correct because prevent coercion, favoritism, and role confusion. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M05 - Patient Rights, Consent, Advocacy, and Ethical Boundaries: Which answer best reflects conflict of interest for an RN case manager in this course?
- Answer: A - Must be recognized when nonclinical pressures compete with patient welfare
- Rationale: Answer A is correct because must be recognized when nonclinical pressures compete with patient welfare. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.

Moodle quiz questions for this module:

QUESTION NAME	TOPIC AREA	DIFFICULTY
M05_Q01_PATIENT_AUTONOMY	patient autonomy	foundational
M05_Q02_CONSENT_AWARE_SHARING	consent-aware sharing	foundational
M05_Q03_ADVOCACY	advocacy	foundational
M05_Q04_ETHICAL_BOUNDARIES	ethical boundaries	foundational
M05_Q05_CONFLICT_OF_INTEREST	conflict of interest	foundational
M05_Q06_REFUSAL_OF_SERVICES	refusal of services	applied
M05_Q07_CULTURAL_HUMILITY	cultural humility	applied
M05_Q08_LANGUAGE_ACCESS	language access	applied
M05_Q09_PRIVACY	privacy	applied
M05_Q10_PROFESSIONAL_COMMUNICATION	professional communication	applied
M05_Q11_ETHICAL_ESCALATION	ethical escalation	applied
M05_Q12_DOCUMENTATION	documentation	advanced
M05_Q13_COURSE_APPROVAL	course approval	advanced
M05_Q14_CONTACT_HOUR_TERMINOLOGY	contact-hour terminology	advanced
M05_Q15_COMPLETION_ATTESTATION	completion attestation	advanced

Mo6 - TRANSITIONS OF CARE, DISCHARGE PLANNING, AND COMMUNITY RESOURCES

Feedback/Attestation: Learner Reflection

Prompt: Describe one decision from this module that changed how you would coordinate care for a patient. Identify the patient-care risk, the RN case management action, and the documentation or escalation step that supports safe follow-up.

Attestation statement: I attest that I completed the required module activities myself, reviewed the learner content, participated in the active interaction, completed the documentation/application exercise, and answered the knowledge check questions.

Completion Gate: Required completion

Completion rule: Mark the module complete only after the learner views the overview and lesson pages, completes the scenario activity, submits the assignment, passes the knowledge check quiz, and submits the reflection/attestation.

Evidence Generated By Moodle

- Page completion logs.
- Lesson/H5P or interaction completion report.
- Assignment submission timestamp and rubric grade.
- Quiz attempt, score, item responses, and pass status.
- Reflection/attestation timestamp.
- Completion gate status.

Instructor/Admin Notes

Review documentation exercises for RN scope, patient-care relevance, factual language, and escalation accuracy. Do not insert provider approval language or certificate release settings until BRN CEP approval metadata is available. If instructor-specific feedback is provided, retain it with course records according to provider policy.

Compliance Guardrails

- Keep all build and learner-facing status as Draft / Pending BRN CEP Approval until approval is issued.
- Use contact hours as the primary measurement.
- Do not represent this content as BRN-approved before provider approval is issued.
- Do not copy official BRN forms, government form text, signer packets, or approval signatures into this Moodle content folder.
- Do not issue a certificate unless all completion, assessment, attestation, and provider-metadata rules are satisfied.

BRN 30 CONTACT-HOUR COURSE MATERIALS

Page: Module Overview

Module ID: M06

Module title: Transitions of Care, Discharge Planning, and Community Resources

Contact-hour target: 3 contact hours.

Designed Moodle time support: approximately 170 learner minutes across lesson reading, case scenario decisions, active interaction, documentation exercise, knowledge check quiz, and reflection. The contact-hour target remains 3 contact hours.

Module purpose: Safe transitions, discharge planning, readmission risk reduction, medication and appointment coordination, caregiver education, and community resource linkage.

Learning objectives:

- Differentiate identify transition risks after hospital, skilled nursing, clinic, and home health changes in an RN case management context.
- Apply coordinate discharge instructions, medication changes, appointments, and equipment needs to realistic direct or indirect patient-care decisions.
- Document teach patients and caregivers warning signs and follow-up expectations using factual, audit-ready nursing language.
- Select appropriate coordination, communication, and escalation steps for connect community resources to clinical and functional barriers.
- Evaluate learner decisions against scope, patient safety, and Draft / Pending BRN CEP Approval compliance guardrails.

Page: Lesson Content

Lesson Segment 1: Identify Transition Risks After Hospital, Skilled Nursing, Clinic, And Home Health Changes

In this segment, the learner studies how to identify transition risks after hospital, skilled nursing, clinic, and home health changes while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Transitions of Care, Discharge Planning, and Community Resources, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When coordinate discharge instructions, medication changes, appointments, and equipment needs is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 2: Coordinate Discharge Instructions, Medication Changes, Appointments, And Equipment Needs

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In this segment, the learner studies how to coordinate discharge instructions, medication changes, appointments, and equipment needs while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Transitions of Care, Discharge Planning, and Community Resources, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When teach patients and caregivers warning signs and follow-up expectations is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 3: Teach Patients And Caregivers Warning Signs And Follow-Up Expectations

In this segment, the learner studies how to teach patients and caregivers warning signs and follow-up expectations while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Transitions of Care, Discharge Planning, and Community Resources, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When connect community resources to clinical and functional barriers is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 4: Connect Community Resources To Clinical And Functional Barriers

In this segment, the learner studies how to connect community resources to clinical and functional barriers while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Transitions of Care, Discharge Planning, and Community Resources, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When use closed-loop follow-up during high-risk transition periods is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

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Lesson Segment 5: Use Closed-Loop Follow-Up During High-Risk Transition Periods

In this segment, the learner studies how to use closed-loop follow-up during high-risk transition periods while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Transitions of Care, Discharge Planning, and Community Resources, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When document transition planning in a way that supports continuity and audit review is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Practice Integration

The module closes by asking the learner to combine the lesson content with the scenario activity and documentation exercise. Learners should be able to explain how their decisions protect patient safety, respect patient rights, support continuity of care, and preserve scope boundaries. The activity is designed for Moodle delivery through a page lesson, case scenario, assignment, quiz, and attestation. Together these activities reasonably support the 3 contact-hour target through reading, applied decision-making, documentation practice, knowledge checks, and reflection.

Lesson/H5P: Case Scenario

A patient returns home after pneumonia with oxygen, new inhalers, a follow-up appointment in seven days, and limited transportation. The caregiver is anxious and the discharge summary has conflicting medication instructions. The RN case manager must stabilize the transition plan and prevent avoidable readmission risk.

Learner task: Review the case, identify the highest-priority patient-care risks, choose the RN case management actions that fit the module objectives, and explain what must be documented or escalated. The Moodle builder may configure this as an H5P branching scenario, Moodle Lesson, or case-response activity.

Expected case output:

- One priority patient-care risk.
- One patient-centered goal.
- Two RN case management actions.
- One communication or escalation step.
- One documentation element that must appear in the record.

Active Learner Interaction

Transition checklist build: learners assemble a 72-hour transition plan and identify which items require immediate escalation.

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Moodle build note: Require learner submission or completion tracking. The interaction should require the learner to make choices, not only read text.

Assignment: Documentation/Application Exercise

Create a discharge follow-up plan that includes medication discrepancy handling, appointment confirmation, equipment status, caregiver teaching, community-resource referrals, and escalation triggers.

Submission format: Moodle online text, file upload, or structured worksheet. Require original learner work and retain timestamped submission evidence.

Rubric criteria:

- Patient-care relevance and RN scope alignment.
- Clear priority setting.
- Factual, complete, and audit-ready documentation.
- Appropriate communication and escalation.
- Compliance with pending-approval and contact-hour terminology guardrails.

Quiz: Knowledge Check

Configure the Moodle quiz from the matching CSV rows in 03_BRN_QUESTION_BANK_MOODLE_IMPORT_READY.csv. Minimum quiz size for this module: 15 multiple-choice questions. Recommended passing score: 80 percent, with retake allowed according to provider policy.

Knowledge checks with answers and rationales:

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- M06 - Transitions of Care, Discharge Planning, and Community Resources: Which answer best reflects transition risk for an RN case manager in this course?
- Answer: A - Increases when medication changes, equipment needs, symptoms, or follow-up barriers are unresolved
- Rationale: Answer A is correct because increases when medication changes, equipment needs, symptoms, or follow-up barriers are unresolved. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M06 - Transitions of Care, Discharge Planning, and Community Resources: Which answer best reflects discharge reconciliation for an RN case manager in this course?
- Answer: A - Compares discharge instructions with current patient use and available orders
- Rationale: Answer A is correct because compares discharge instructions with current patient use and available orders. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M06 - Transitions of Care, Discharge Planning, and Community Resources: Which answer best reflects caregiver teaching for an RN case manager in this course?
- Answer: A - Includes warning signs, who to call, and teach-back of key instructions
- Rationale: Answer A is correct because includes warning signs, who to call, and teach-back of key instructions. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M06 - Transitions of Care, Discharge Planning, and Community Resources: Which answer best reflects appointment coordination for an RN case manager in this course?
- Answer: A - Confirms date, transportation, records needed, and patient understanding
- Rationale: Answer A is correct because confirms date, transportation, records needed, and patient understanding. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M06 - Transitions of Care, Discharge Planning, and Community Resources: Which answer best reflects equipment access for an RN case manager in this course?
- Answer: A - Must be verified when safety or treatment depends on it
- Rationale: Answer A is correct because must be verified when safety or treatment depends on it. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.

Moodle quiz questions for this module:

QUESTION NAME	TOPIC AREA	DIFFICULTY
M06_Q01_TRANSITION_RISK	transition risk	foundational
M06_Q02_DISCHARGE_RECONCILIATION	discharge reconciliation	foundational
M06_Q03_CAREGIVER_TEACHING	caregiver teaching	foundational
M06_Q04_APPOINTMENT_COORDINATION	appointment coordination	foundational
M06_Q05_EQUIPMENT_ACCESS	equipment access	foundational
M06_Q06_COMMUNITY_RESOURCES	community resources	applied
M06_Q07_READMISSION_PREVENTION	readmission prevention	applied
M06_Q08_FOLLOW_UP_TIMING	follow-up timing	applied
M06_Q09_CLOSED_LOOP_REFERRAL	closed-loop referral	applied
M06_Q10_PATIENT_GOALS	patient goals	applied
M06_Q11_DOCUMENTATION	documentation	applied
M06_Q12_URGENT_ESCALATION	urgent escalation	advanced
M06_Q13_MOODLE_COMPLETION	Moodle completion	advanced
M06_Q14_DRAFT_STATUS	draft status	advanced
M06_Q15_CONTACT_HOURS	contact hours	advanced

Mo7 - UTILIZATION REVIEW, MEDICAL NECESSITY, AND PAYER-SAFE COMMUNICATION

Feedback/Attestation: Learner Reflection

Prompt: Describe one decision from this module that changed how you would coordinate care for a patient. Identify the patient-care risk, the RN case management action, and the documentation or escalation step that supports safe follow-up.

Attestation statement: I attest that I completed the required module activities myself, reviewed the learner content, participated in the active interaction, completed the documentation/application exercise, and answered the knowledge check questions.

Completion Gate: Required completion

Completion rule: Mark the module complete only after the learner views the overview and lesson pages, completes the scenario activity, submits the assignment, passes the knowledge check quiz, and submits the reflection/attestation.

Evidence Generated By Moodle

- Page completion logs.
- Lesson/H5P or interaction completion report.
- Assignment submission timestamp and rubric grade.
- Quiz attempt, score, item responses, and pass status.
- Reflection/attestation timestamp.
- Completion gate status.

Instructor/Admin Notes

Review documentation exercises for RN scope, patient-care relevance, factual language, and escalation accuracy. Do not insert provider approval language or certificate release settings until BRN CEP approval metadata is available. If instructor-specific feedback is provided, retain it with course records according to provider policy.

Compliance Guardrails

- Keep all build and learner-facing status as Draft / Pending BRN CEP Approval until approval is issued.
- Use contact hours as the primary measurement.
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- Do not issue a certificate unless all completion, assessment, attestation, and provider-metadata rules are satisfied.

BRN 30 CONTACT-HOUR COURSE MATERIALS

Page: Module Overview

Module ID: M07

Module title: Utilization Review, Medical Necessity, and Payer-Safe Communication

Contact-hour target: 3 contact hours.

Designed Moodle time support: approximately 170 learner minutes across lesson reading, case scenario decisions, active interaction, documentation exercise, knowledge check quiz, and reflection. The contact-hour target remains 3 contact hours.

Module purpose: RN communication about medical necessity, utilization review concepts, coverage-sensitive language, and documentation that supports care without making inappropriate guarantees.

Learning objectives:

- Differentiate explain utilization review concepts from an RN case management perspective in an RN case management context.
- Apply connect medical necessity communication to clinical facts and patient needs to realistic direct or indirect patient-care decisions.
- Document avoid promising coverage, payment, authorization, or approval outcomes using factual, audit-ready nursing language.
- Select appropriate coordination, communication, and escalation steps for prepare payer-safe updates that are accurate, concise, and clinically grounded.
- Evaluate learner decisions against scope, patient safety, and Draft / Pending BRN CEP Approval compliance guardrails.

Page: Lesson Content

Lesson Segment 1: Explain Utilization Review Concepts From An Rn Case Management Perspective

In this segment, the learner studies how to explain utilization review concepts from an RN case management perspective while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Utilization Review, Medical Necessity, and Payer-Safe Communication, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When connect medical necessity communication to clinical facts and patient needs is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 2: Connect Medical Necessity Communication To Clinical Facts And Patient Needs

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In this segment, the learner studies how to connect medical necessity communication to clinical facts and patient needs while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Utilization Review, Medical Necessity, and Payer-Safe Communication, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When avoid promising coverage, payment, authorization, or approval outcomes is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 3: Avoid Promising Coverage, Payment, Authorization, Or Approval Outcomes

In this segment, the learner studies how to avoid promising coverage, payment, authorization, or approval outcomes while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Utilization Review, Medical Necessity, and Payer-Safe Communication, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When prepare payer-safe updates that are accurate, concise, and clinically grounded is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 4: Prepare Payer-Safe Updates That Are Accurate, Concise, And Clinically Grounded

In this segment, the learner studies how to prepare payer-safe updates that are accurate, concise, and clinically grounded while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Utilization Review, Medical Necessity, and Payer-Safe Communication, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When identify when payer questions require supervisor, compliance, or provider input is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

BRN 30 CONTACT-HOUR COURSE MATERIALS

Lesson Segment 5: Identify When Payer Questions Require Supervisor, Compliance, Or Provider Input

In this segment, the learner studies how to identify when payer questions require supervisor, compliance, or provider input while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Utilization Review, Medical Necessity, and Payer-Safe Communication, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When document utilization-related communication without overstating certainty is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Practice Integration

The module closes by asking the learner to combine the lesson content with the scenario activity and documentation exercise. Learners should be able to explain how their decisions protect patient safety, respect patient rights, support continuity of care, and preserve scope boundaries. The activity is designed for Moodle delivery through a page lesson, case scenario, assignment, quiz, and attestation. Together these activities reasonably support the 3 contact-hour target through reading, applied decision-making, documentation practice, knowledge checks, and reflection.

Lesson/H5P: Case Scenario

A payer requests additional information before authorizing continued home health visits. The patient still has wound-care needs, mobility limitations, and caregiver gaps. The RN case manager must communicate clinical facts and risks without promising coverage or using language that exceeds the RN role.

Learner task: Review the case, identify the highest-priority patient-care risks, choose the RN case management actions that fit the module objectives, and explain what must be documented or escalated. The Moodle builder may configure this as an H5P branching scenario, Moodle Lesson, or case-response activity.

Expected case output:

- One priority patient-care risk.
- One patient-centered goal.
- Two RN case management actions.
- One communication or escalation step.
- One documentation element that must appear in the record.

Active Learner Interaction

Payer-safe wording drill: learners rewrite risky phrases into clinical, neutral, documented communication.

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Moodle build note: Require learner submission or completion tracking. The interaction should require the learner to make choices, not only read text.

Assignment: Documentation/Application Exercise

Draft a payer communication note that summarizes current clinical status, skilled need indicators, safety risks, patient response, and requested follow-up without guaranteeing payment or approval.

Submission format: Moodle online text, file upload, or structured worksheet. Require original learner work and retain timestamped submission evidence.

Rubric criteria:

- Patient-care relevance and RN scope alignment.
- Clear priority setting.
- Factual, complete, and audit-ready documentation.
- Appropriate communication and escalation.
- Compliance with pending-approval and contact-hour terminology guardrails.

Quiz: Knowledge Check

Configure the Moodle quiz from the matching CSV rows in 03_BRN_QUESTION_BANK_MOODLE_IMPORT_READY.csv. Minimum quiz size for this module: 15 multiple-choice questions. Recommended passing score: 80 percent, with retake allowed according to provider policy.

Knowledge checks with answers and rationales:

BRN 30 CONTACT-HOUR COURSE MATERIALS

- M07 - Utilization Review, Medical Necessity, and Payer-Safe Communication: Which answer best reflects medical necessity communication for an RN case manager in this course?
- Answer: A - Uses patient-specific clinical facts, functional needs, risks, and skilled interventions
- Rationale: Answer A is correct because uses patient-specific clinical facts, functional needs, risks, and skilled interventions. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M07 - Utilization Review, Medical Necessity, and Payer-Safe Communication: Which answer best reflects payer-safe language for an RN case manager in this course?
- Answer: A - Does not promise authorization, coverage, reimbursement, or approval
- Rationale: Answer A is correct because does not promise authorization, coverage, reimbursement, or approval. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M07 - Utilization Review, Medical Necessity, and Payer-Safe Communication: Which answer best reflects utilization review for an RN case manager in this course?
- Answer: A - Evaluates the fit between patient need, service use, criteria, and documentation
- Rationale: Answer A is correct because evaluates the fit between patient need, service use, criteria, and documentation. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M07 - Utilization Review, Medical Necessity, and Payer-Safe Communication: Which answer best reflects clinical summary for an RN case manager in this course?
- Answer: A - Should be concise, objective, and linked to current assessment findings
- Rationale: Answer A is correct because should be concise, objective, and linked to current assessment findings. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M07 - Utilization Review, Medical Necessity, and Payer-Safe Communication: Which answer best reflects coverage question for an RN case manager in this course?
- Answer: A - May require referral to payer, supervisor, or compliance rather than RN speculation
- Rationale: Answer A is correct because may require referral to payer, supervisor, or compliance rather than RN speculation. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.

Moodle quiz questions for this module:

BRN 30 CONTACT-HOUR COURSE MATERIALS

QUESTION NAME	TOPIC AREA	DIFFICULTY
M07_Q01_MEDICAL_NECESSITY_COMMUNICATION	medical necessity communication	foundational
M07_Q02_PAYER_SAFE_LANGUAGE	payer-safe language	foundational
M07_Q03_UTILIZATION_REVIEW	utilization review	foundational
M07_Q04_CLINICAL_SUMMARY	clinical summary	foundational
M07_Q05_COVERAGE_QUESTION	coverage question	foundational
M07_Q06_AUTHORIZATION_DELAY	authorization delay	applied
M07_Q07_SKILLED_NEED	skilled need	applied
M07_Q08_OVERSTATEMENT_RISK	overstatement risk	applied
M07_Q09_PATIENT_COMMUNICATION	patient communication	applied
M07_Q10_PROVIDER_INPUT	provider input	applied
M07_Q11_DOCUMENTATION_TRAIL	documentation trail	applied
M07_Q12_ETHICAL_COMMUNICATION	ethical communication	advanced
M07_Q13_BRN_APPROVAL_STATUS	BRN approval status	advanced
M07_Q14_CONTACT_HOUR_WORDING	contact-hour wording	advanced
M07_Q15_QUIZ_EVIDENCE	quiz evidence	advanced

Feedback/Attestation: Learner Reflection

Prompt: Describe one decision from this module that changed how you would coordinate care for a patient. Identify the patient-care risk, the RN case management action, and the documentation or escalation step that supports safe follow-up.

Attestation statement: I attest that I completed the required module activities myself, reviewed the learner content, participated in the active interaction, completed the documentation/application exercise, and answered the knowledge check questions.

Completion Gate: Required completion

Completion rule: Mark the module complete only after the learner views the overview and lesson pages, completes the scenario activity, submits the assignment, passes the knowledge check quiz, and submits the reflection/attestation.

Evidence Generated By Moodle

- Page completion logs.
- Lesson/H5P or interaction completion report.
- Assignment submission timestamp and rubric grade.
- Quiz attempt, score, item responses, and pass status.
- Reflection/attestation timestamp.
- Completion gate status.

Instructor/Admin Notes

Review documentation exercises for RN scope, patient-care relevance, factual language, and escalation accuracy. Do not insert provider approval language or certificate release settings until BRN CEP approval metadata is available. If instructor-specific feedback is provided, retain it with course records according to provider policy.

Mo8 - QUALITY ASSURANCE, RISK MANAGEMENT, INCIDENT REPORTING, AND ESCALATION

Compliance Guardrails

- Keep all build and learner-facing status as Draft / Pending BRN CEP Approval until approval is issued.
- Use contact hours as the primary measurement.
- Do not represent this content as BRN-approved before provider approval is issued.
- Do not copy official BRN forms, government form text, signer packets, or approval signatures into this Moodle content folder.
- Do not issue a certificate unless all completion, assessment, attestation, and provider-metadata rules are satisfied.

Page: Module Overview

Module ID: M08

Module title: Quality Assurance, Risk Management, Incident Reporting, and Escalation

Contact-hour target: 3 contact hours.

Designed Moodle time support: approximately 170 learner minutes across lesson reading, case scenario decisions, active interaction, documentation exercise, knowledge check quiz, and reflection. The contact-hour target remains 3 contact hours.

Module purpose: Quality assurance, incident recognition, risk triage, escalation pathways, root-cause thinking, and improvement documentation within RN case management.

Learning objectives:

- Differentiate recognize incidents, near misses, complaints, and quality concerns during case management in an RN case management context.
- Apply use escalation pathways based on urgency and patient safety risk to realistic direct or indirect patient-care decisions.
- Document document factual incident information without blame or unsupported conclusions using factual, audit-ready nursing language.
- Select appropriate coordination, communication, and escalation steps for participate in quality review by identifying process gaps and prevention actions.
- Evaluate learner decisions against scope, patient safety, and Draft / Pending BRN CEP Approval compliance guardrails.

Page: Lesson Content

Lesson Segment 1: Recognize Incidents, Near Misses, Complaints, And Quality Concerns During Case Management

BRN 30 CONTACT-HOUR COURSE MATERIALS

In this segment, the learner studies how to recognize incidents, near misses, complaints, and quality concerns during case management while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Quality Assurance, Risk Management, Incident Reporting, and Escalation, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When use escalation pathways based on urgency and patient safety risk is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 2: Use Escalation Pathways Based On Urgency And Patient Safety Risk

In this segment, the learner studies how to use escalation pathways based on urgency and patient safety risk while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Quality Assurance, Risk Management, Incident Reporting, and Escalation, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When document factual incident information without blame or unsupported conclusions is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 3: Document Factual Incident Information Without Blame Or Unsupported Conclusions

In this segment, the learner studies how to document factual incident information without blame or unsupported conclusions while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Quality Assurance, Risk Management, Incident Reporting, and Escalation, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When participate in quality review by identifying process gaps and prevention actions is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

BRN 30 CONTACT-HOUR COURSE MATERIALS

Lesson Segment 4: Participate In Quality Review By Identifying Process Gaps And Prevention Actions

In this segment, the learner studies how to participate in quality review by identifying process gaps and prevention actions while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Quality Assurance, Risk Management, Incident Reporting, and Escalation, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When distinguish routine coordination barriers from reportable or urgent risk events is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 5: Distinguish Routine Coordination Barriers From Reportable Or Urgent Risk Events

In this segment, the learner studies how to distinguish routine coordination barriers from reportable or urgent risk events while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Quality Assurance, Risk Management, Incident Reporting, and Escalation, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When protect patient safety while following organizational policy and confidentiality controls is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Practice Integration

The module closes by asking the learner to combine the lesson content with the scenario activity and documentation exercise. Learners should be able to explain how their decisions protect patient safety, respect patient rights, support continuity of care, and preserve scope boundaries. The activity is designed for Moodle delivery through a page lesson, case scenario, assignment, quiz, and attestation. Together these activities reasonably support the 3 contact-hour target through reading, applied decision-making, documentation practice, knowledge checks, and reflection.

Lesson/H5P: Case Scenario

BRN 30 CONTACT-HOUR COURSE MATERIALS

During a follow-up call, an RN case manager learns that a patient fell two days after discharge and did not report it because they feared losing independence. The medication list also shows a sedating medication taken at the wrong time. The RN must address immediate safety, reporting, documentation, and prevention.

Learner task: Review the case, identify the highest-priority patient-care risks, choose the RN case management actions that fit the module objectives, and explain what must be documented or escalated. The Moodle builder may configure this as an H5P branching scenario, Moodle Lesson, or case-response activity.

Expected case output:

- One priority patient-care risk.
- One patient-centered goal.
- Two RN case management actions.
- One communication or escalation step.
- One documentation element that must appear in the record.

Active Learner Interaction

Escalation pathway map: learners choose immediate, same-day, routine, and quality-review pathways for sample events.

Moodle build note: Require learner submission or completion tracking. The interaction should require the learner to make choices, not only read text.

Assignment: Documentation/Application Exercise

Prepare an incident follow-up note and quality-improvement worksheet that identifies factual event details, immediate actions, escalation contacts, contributing factors, and prevention steps.

Submission format: Moodle online text, file upload, or structured worksheet. Require original learner work and retain timestamped submission evidence.

Rubric criteria:

- Patient-care relevance and RN scope alignment.
- Clear priority setting.
- Factual, complete, and audit-ready documentation.
- Appropriate communication and escalation.
- Compliance with pending-approval and contact-hour terminology guardrails.

Quiz: Knowledge Check

Configure the Moodle quiz from the matching CSV rows in 03_BRN_QUESTION_BANK_MOODLE_IMPORT_READY.csv. Minimum quiz size for this module: 15 multiple-choice questions. Recommended passing score: 80 percent, with retake allowed according to provider policy.

Knowledge checks with answers and rationales:

BRN 30 CONTACT-HOUR COURSE MATERIALS

- M08 - Quality Assurance, Risk Management, Incident Reporting, and Escalation: Which answer best reflects incident recognition for an RN case manager in this course?
- Answer: A - Includes events, near misses, complaints, and conditions that could harm the patient
- Rationale: Answer A is correct because includes events, near misses, complaints, and conditions that could harm the patient. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M08 - Quality Assurance, Risk Management, Incident Reporting, and Escalation: Which answer best reflects urgent escalation for an RN case manager in this course?
- Answer: A - Is based on current risk, change in condition, and policy requirements
- Rationale: Answer A is correct because is based on current risk, change in condition, and policy requirements. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M08 - Quality Assurance, Risk Management, Incident Reporting, and Escalation: Which answer best reflects factual reporting for an RN case manager in this course?
- Answer: A - Documents observed or reported facts without blame
- Rationale: Answer A is correct because documents observed or reported facts without blame. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M08 - Quality Assurance, Risk Management, Incident Reporting, and Escalation: Which answer best reflects root-cause thinking for an RN case manager in this course?
- Answer: A - Looks for process contributors rather than stopping at individual fault
- Rationale: Answer A is correct because looks for process contributors rather than stopping at individual fault. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M08 - Quality Assurance, Risk Management, Incident Reporting, and Escalation: Which answer best reflects quality improvement for an RN case manager in this course?
- Answer: A - Uses patterns and prevention actions to reduce recurrence
- Rationale: Answer A is correct because uses patterns and prevention actions to reduce recurrence. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.

Moodle quiz questions for this module:

QUESTION NAME	TOPIC AREA	DIFFICULTY
M08_Q01_INCIDENT_RECOGNITION	incident recognition	foundational
M08_Q02_URGENT_ESCALATION	urgent escalation	foundational
M08_Q03_FACTUAL_REPORTING	factual reporting	foundational
M08_Q04_ROOT_CAUSE_THINKING	root-cause thinking	foundational
M08_Q05_QUALITY_IMPROVEMENT	quality improvement	foundational
M08_Q06_FALL_RISK	fall risk	applied
M08_Q07_MEDICATION_SAFETY	medication safety	applied
M08_Q08_COMPLAINT_HANDLING	complaint handling	applied
M08_Q09_NEAR_MISS	near miss	applied
M08_Q10_CONFIDENTIALITY	confidentiality	applied
M08_Q11_FOLLOW_UP_EVIDENCE	follow-up evidence	applied
M08_Q12_ROUTINE_BARRIER	routine barrier	advanced
M08_Q13_COMPLETION_RULE	completion rule	advanced
M08_Q14_PENDING_APPROVAL_LABEL	pending approval label	advanced
M08_Q15_CONTACT_HOURS	contact hours	advanced

Mo9 - CHRONIC DISEASE, GERIATRIC, AND HIGH-RISK PATIENT MANAGEMENT

Feedback/Attestation: Learner Reflection

Prompt: Describe one decision from this module that changed how you would coordinate care for a patient. Identify the patient-care risk, the RN case management action, and the documentation or escalation step that supports safe follow-up.

Attestation statement: I attest that I completed the required module activities myself, reviewed the learner content, participated in the active interaction, completed the documentation/application exercise, and answered the knowledge check questions.

Completion Gate: Required completion

Completion rule: Mark the module complete only after the learner views the overview and lesson pages, completes the scenario activity, submits the assignment, passes the knowledge check quiz, and submits the reflection/attestation.

Evidence Generated By Moodle

- Page completion logs.
- Lesson/H5P or interaction completion report.
- Assignment submission timestamp and rubric grade.
- Quiz attempt, score, item responses, and pass status.
- Reflection/attestation timestamp.
- Completion gate status.

Instructor/Admin Notes

Review documentation exercises for RN scope, patient-care relevance, factual language, and escalation accuracy. Do not insert provider approval language or certificate release settings until BRN CEP approval metadata is available. If instructor-specific feedback is provided, retain it with course records according to provider policy.

Compliance Guardrails

- Keep all build and learner-facing status as Draft / Pending BRN CEP Approval until approval is issued.
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- Do not issue a certificate unless all completion, assessment, attestation, and provider-metadata rules are satisfied.

BRN 30 CONTACT-HOUR COURSE MATERIALS

Page: Module Overview

Module ID: M09

Module title: Chronic Disease, Geriatric, and High-Risk Patient Management

Contact-hour target: 3 contact hours.

Designed Moodle time support: approximately 170 learner minutes across lesson reading, case scenario decisions, active interaction, documentation exercise, knowledge check quiz, and reflection. The contact-hour target remains 3 contact hours.

Module purpose: RN case management for patients with chronic disease, geriatric needs, polypharmacy, frailty, cognitive concerns, social risk, and recurrent utilization.

Learning objectives:

- Differentiate identify high-risk patient characteristics that affect care coordination intensity in an RN case management context.
- Apply coordinate chronic disease teaching, monitoring, medication follow-up, and provider communication to realistic direct or indirect patient-care decisions.
- Document address geriatric syndromes such as falls, frailty, cognition, nutrition, and caregiver strain using factual, audit-ready nursing language.
- Select appropriate coordination, communication, and escalation steps for adapt plans for patients with multiple conditions and competing priorities.
- Evaluate learner decisions against scope, patient safety, and Draft / Pending BRN CEP Approval compliance guardrails.

Page: Lesson Content

Lesson Segment 1: Identify High-Risk Patient Characteristics That Affect Care Coordination Intensity

In this segment, the learner studies how to identify high-risk patient characteristics that affect care coordination intensity while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Chronic Disease, Geriatric, and High-Risk Patient Management, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When coordinate chronic disease teaching, monitoring, medication follow-up, and provider communication is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 2: Coordinate Chronic Disease Teaching, Monitoring, Medication Follow-Up, And Provider Communication

BRN 30 CONTACT-HOUR COURSE MATERIALS

In this segment, the learner studies how to coordinate chronic disease teaching, monitoring, medication follow-up, and provider communication while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Chronic Disease, Geriatric, and High-Risk Patient Management, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When address geriatric syndromes such as falls, frailty, cognition, nutrition, and caregiver strain is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 3: Address Geriatric Syndromes Such As Falls, Frailty, Cognition, Nutrition, And Caregiver Strain

In this segment, the learner studies how to address geriatric syndromes such as falls, frailty, cognition, nutrition, and caregiver strain while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Chronic Disease, Geriatric, and High-Risk Patient Management, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When adapt plans for patients with multiple conditions and competing priorities is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 4: Adapt Plans For Patients With Multiple Conditions And Competing Priorities

In this segment, the learner studies how to adapt plans for patients with multiple conditions and competing priorities while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Chronic Disease, Geriatric, and High-Risk Patient Management, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When use risk stratification to set follow-up frequency and escalation triggers is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

BRN 30 CONTACT-HOUR COURSE MATERIALS

Lesson Segment 5: Use Risk Stratification To Set Follow-Up Frequency And Escalation Triggers

In this segment, the learner studies how to use risk stratification to set follow-up frequency and escalation triggers while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Chronic Disease, Geriatric, and High-Risk Patient Management, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When document high-risk plans with measurable goals and evidence of patient response is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Practice Integration

The module closes by asking the learner to combine the lesson content with the scenario activity and documentation exercise. Learners should be able to explain how their decisions protect patient safety, respect patient rights, support continuity of care, and preserve scope boundaries. The activity is designed for Moodle delivery through a page lesson, case scenario, assignment, quiz, and attestation. Together these activities reasonably support the 3 contact-hour target through reading, applied decision-making, documentation practice, knowledge checks, and reflection.

Lesson/H5P: Case Scenario

An 82-year-old patient has heart failure, diabetes, mild cognitive impairment, recent falls, and a caregiver who is overwhelmed. The patient has missed appointments and is unsure how to use a new glucometer. The RN case manager must prioritize risks, coordinate support, and create a realistic follow-up plan.

Learner task: Review the case, identify the highest-priority patient-care risks, choose the RN case management actions that fit the module objectives, and explain what must be documented or escalated. The Moodle builder may configure this as an H5P branching scenario, Moodle Lesson, or case-response activity.

Expected case output:

- One priority patient-care risk.
- One patient-centered goal.
- Two RN case management actions.
- One communication or escalation step.
- One documentation element that must appear in the record.

Active Learner Interaction

High-risk care plan conference: learners choose top risks, follow-up timing, team partners, and patient education priorities for a complex profile.

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Moodle build note: Require learner submission or completion tracking. The interaction should require the learner to make choices, not only read text.

Assignment: Documentation/Application Exercise

Build a high-risk case management plan that includes risk stratification, chronic disease priorities, geriatric concerns, caregiver supports, teaching plan, and measurable follow-up outcomes.

Submission format: Moodle online text, file upload, or structured worksheet. Require original learner work and retain timestamped submission evidence.

Rubric criteria:

- Patient-care relevance and RN scope alignment.
- Clear priority setting.
- Factual, complete, and audit-ready documentation.
- Appropriate communication and escalation.
- Compliance with pending-approval and contact-hour terminology guardrails.

Quiz: Knowledge Check

Configure the Moodle quiz from the matching CSV rows in 03_BRN_QUESTION_BANK_MOODLE_IMPORT_READY.csv. Minimum quiz size for this module: 15 multiple-choice questions. Recommended passing score: 80 percent, with retake allowed according to provider policy.

Knowledge checks with answers and rationales:

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- M09 - Chronic Disease, Geriatric, and High-Risk Patient Management: Which answer best reflects risk stratification for an RN case manager in this course?
- Answer: A - Uses clinical instability, functional limits, social barriers, and utilization history
- Rationale: Answer A is correct because uses clinical instability, functional limits, social barriers, and utilization history. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M09 - Chronic Disease, Geriatric, and High-Risk Patient Management: Which answer best reflects polypharmacy risk for an RN case manager in this course?
- Answer: A - Requires medication reconciliation and attention to adverse effects
- Rationale: Answer A is correct because requires medication reconciliation and attention to adverse effects. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M09 - Chronic Disease, Geriatric, and High-Risk Patient Management: Which answer best reflects geriatric falls for an RN case manager in this course?
- Answer: A - Require assessment of mobility, environment, medications, cognition, and follow-up
- Rationale: Answer A is correct because require assessment of mobility, environment, medications, cognition, and follow-up. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M09 - Chronic Disease, Geriatric, and High-Risk Patient Management: Which answer best reflects chronic disease teaching for an RN case manager in this course?
- Answer: A - Uses patient-specific warning signs and teach-back
- Rationale: Answer A is correct because uses patient-specific warning signs and teach-back. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M09 - Chronic Disease, Geriatric, and High-Risk Patient Management: Which answer best reflects caregiver strain for an RN case manager in this course?
- Answer: A - Can affect adherence, safety, and transition success
- Rationale: Answer A is correct because can affect adherence, safety, and transition success. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.

Moodle quiz questions for this module:

QUESTION NAME	TOPIC AREA	DIFFICULTY
M09_Q01_RISK_STRATIFICATION	risk stratification	foundational
M09_Q02_POLYPHARMACY_RISK	polypharmacy risk	foundational
M09_Q03_GERIATRIC_FALLS	geriatric falls	foundational
M09_Q04_CHRONIC_DISEASE_TEACHING	chronic disease teaching	foundational
M09_Q05_CAREGIVER_STRAIN	caregiver strain	foundational
M09_Q06_COGNITIVE_IMPAIRMENT	cognitive impairment	applied
M09_Q07_MULTIPLE_CONDITIONS	multiple conditions	applied
M09_Q08_NUTRITION_CONCERN	nutrition concern	applied
M09_Q09_FOLLOW_UP_INTENSITY	follow-up intensity	applied
M09_Q10_PROVIDER_COMMUNICATION	provider communication	applied
M09_Q11_PATIENT_GOALS	patient goals	applied
M09_Q12_MEASURABLE_OUTCOMES	measurable outcomes	advanced
M09_Q13_COURSE_STATUS	course status	advanced
M09_Q14_CONTACT_HOUR_TRACKING	contact-hour tracking	advanced
M09_Q15_FINAL_READINESS	final readiness	advanced

M10 – INTEGRATED CASE MANAGEMENT CAPSTONE AND FINAL ASSESSMENT

Feedback/Attestation: Learner Reflection

Prompt: Describe one decision from this module that changed how you would coordinate care for a patient. Identify the patient-care risk, the RN case management action, and the documentation or escalation step that supports safe follow-up.

Attestation statement: I attest that I completed the required module activities myself, reviewed the learner content, participated in the active interaction, completed the documentation/application exercise, and answered the knowledge check questions.

Completion Gate: Required completion

Completion rule: Mark the module complete only after the learner views the overview and lesson pages, completes the scenario activity, submits the assignment, passes the knowledge check quiz, and submits the reflection/attestation.

Evidence Generated By Moodle

- Page completion logs.
- Lesson/H5P or interaction completion report.
- Assignment submission timestamp and rubric grade.
- Quiz attempt, score, item responses, and pass status.
- Reflection/attestation timestamp.
- Completion gate status.

Instructor/Admin Notes

Review documentation exercises for RN scope, patient-care relevance, factual language, and escalation accuracy. Do not insert provider approval language or certificate release settings until BRN CEP approval metadata is available. If instructor-specific feedback is provided, retain it with course records according to provider policy.

Compliance Guardrails

- Keep all build and learner-facing status as Draft / Pending BRN CEP Approval until approval is issued.
- Use contact hours as the primary measurement.
- Do not represent this content as BRN-approved before provider approval is issued.
- Do not copy official BRN forms, government form text, signer packets, or approval signatures into this Moodle content folder.
- Do not issue a certificate unless all completion, assessment, attestation, and provider-metadata rules are satisfied.

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Page: Module Overview

Module ID: M10

Module title: Integrated Case Management Capstone and Final Assessment

Contact-hour target: 3 contact hours.

Designed Moodle time support: approximately 170 learner minutes across lesson reading, case scenario decisions, active interaction, documentation exercise, knowledge check quiz, and reflection. The contact-hour target remains 3 contact hours.

Module purpose: Integrated application of RN case management scope, assessment, coordination, documentation, advocacy, transitions, utilization communication, quality escalation, and high-risk patient management.

Learning objectives:

- Differentiate synthesize assessment findings into a complete RN case management plan in an RN case management context.
- Apply prioritize safety, patient goals, communication, and documentation across a complex case to realistic direct or indirect patient-care decisions.
- Document complete a capstone documentation packet using audit-ready language using factual, audit-ready nursing language.
- Select appropriate coordination, communication, and escalation steps for answer final assessment questions that test application rather than recall only.
- Evaluate learner decisions against scope, patient safety, and Draft / Pending BRN CEP Approval compliance guardrails.

Page: Lesson Content

Lesson Segment 1: Synthesize Assessment Findings Into A Complete Rn Case Management Plan

In this segment, the learner studies how to synthesize assessment findings into a complete RN case management plan while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Integrated Case Management Capstone and Final Assessment, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When prioritize safety, patient goals, communication, and documentation across a complex case is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 2: Prioritize Safety, Patient Goals, Communication, And Documentation Across A Complex Case

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In this segment, the learner studies how to prioritize safety, patient goals, communication, and documentation across a complex case while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Integrated Case Management Capstone and Final Assessment, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When complete a capstone documentation packet using audit-ready language is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 3: Complete A Capstone Documentation Packet Using Audit-Ready Language

In this segment, the learner studies how to complete a capstone documentation packet using audit-ready language while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Integrated Case Management Capstone and Final Assessment, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When answer final assessment questions that test application rather than recall only is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Lesson Segment 4: Answer Final Assessment Questions That Test Application Rather Than Recall Only

In this segment, the learner studies how to answer final assessment questions that test application rather than recall only while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Integrated Case Management Capstone and Final Assessment, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When identify readiness for certificate release based on Moodle completion evidence is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

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Lesson Segment 5: Identify Readiness For Certificate Release Based On Moodle Completion Evidence

In this segment, the learner studies how to identify readiness for certificate release based on Moodle completion evidence while practicing as a registered nurse case manager. The emphasis is not on generic task completion; it is on connecting assessment, nursing judgment, communication, and follow-up to patient care. For Integrated Case Management Capstone and Final Assessment, the RN case manager must ask what patient risk is present, what information is reliable, what action is within RN authority, and what requires another team member or provider. This approach keeps the work tied to direct or indirect patient/client care and avoids reducing case management to administrative tracking.

A strong learner response should show a clear chain from finding to action. The RN identifies the relevant patient need, chooses a coordination step, explains why the step matters, and documents the outcome or next follow-up. When apply compliance guardrails before any BRN approval or provider-number language is used is also present, the learner should avoid isolated decisions. For example, communication, teaching, referral, payer contact, or escalation should be linked to the patient's current condition and goals. The record should allow another qualified reviewer to understand what was known, what was done, who was contacted, what remains pending, and why the RN selected that priority.

Practice Integration

The module closes by asking the learner to combine the lesson content with the scenario activity and documentation exercise. Learners should be able to explain how their decisions protect patient safety, respect patient rights, support continuity of care, and preserve scope boundaries. The activity is designed for Moodle delivery through a page lesson, case scenario, assignment, quiz, and attestation. Together these activities reasonably support the 3 contact-hour target through reading, applied decision-making, documentation practice, knowledge checks, and reflection.

Lesson/H5P: Case Scenario

The final capstone follows a complex patient discharged after sepsis with diabetes, wound care, medication changes, transportation barriers, caregiver stress, and a pending payer review. The learner must create an integrated RN case management plan, communicate with the team, document the record, and decide what must be escalated.

Learner task: Review the case, identify the highest-priority patient-care risks, choose the RN case management actions that fit the module objectives, and explain what must be documented or escalated. The Moodle builder may configure this as an H5P branching scenario, Moodle Lesson, or case-response activity.

Expected case output:

- One priority patient-care risk.
- One patient-centered goal.
- Two RN case management actions.
- One communication or escalation step.
- One documentation element that must appear in the record.

Active Learner Interaction

Capstone decision path: learners complete a branching case with assessment, coordination, documentation, payer communication, ethics, and escalation checkpoints.

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Moodle build note: Require learner submission or completion tracking. The interaction should require the learner to make choices, not only read text.

Assignment: Documentation/Application Exercise

Submit the final capstone packet: integrated assessment summary, prioritized care coordination plan, communication log, documentation note, patient teaching plan, escalation list, and reflection on compliance guardrails.

Submission format: Moodle online text, file upload, or structured worksheet. Require original learner work and retain timestamped submission evidence.

Rubric criteria:

- Patient-care relevance and RN scope alignment.
- Clear priority setting.
- Factual, complete, and audit-ready documentation.
- Appropriate communication and escalation.
- Compliance with pending-approval and contact-hour terminology guardrails.

Quiz: Knowledge Check

Configure the Moodle quiz from the matching CSV rows in 03_BRN_QUESTION_BANK_MOODLE_IMPORT_READY.csv. Minimum quiz size for this module: 15 multiple-choice questions. Recommended passing score: 80 percent, with retake allowed according to provider policy.

Knowledge checks with answers and rationales:

BRN 30 CONTACT-HOUR COURSE MATERIALS

- M10 - Integrated Case Management Capstone and Final Assessment: Which answer best reflects integrated assessment for an RN case manager in this course?
- Answer: A - Combines clinical, functional, psychosocial, medication, and access data
- Rationale: Answer A is correct because combines clinical, functional, psychosocial, medication, and access data. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M10 - Integrated Case Management Capstone and Final Assessment: Which answer best reflects priority setting for an RN case manager in this course?
- Answer: A - Addresses immediate safety and high-risk barriers before lower-risk tasks
- Rationale: Answer A is correct because addresses immediate safety and high-risk barriers before lower-risk tasks. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M10 - Integrated Case Management Capstone and Final Assessment: Which answer best reflects capstone documentation for an RN case manager in this course?
- Answer: A - Must be factual, complete, timely, and linked to nursing actions
- Rationale: Answer A is correct because must be factual, complete, timely, and linked to nursing actions. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M10 - Integrated Case Management Capstone and Final Assessment: Which answer best reflects team communication for an RN case manager in this course?
- Answer: A - Uses closed-loop follow-up and clear ownership
- Rationale: Answer A is correct because uses closed-loop follow-up and clear ownership. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.
- M10 - Integrated Case Management Capstone and Final Assessment: Which answer best reflects patient advocacy for an RN case manager in this course?
- Answer: A - Supports needs while respecting autonomy and scope
- Rationale: Answer A is correct because supports needs while respecting autonomy and scope. The other options either remove the RN case management link to patient care, promise an outcome outside the RN role, or create unsafe documentation practice.

Moodle quiz questions for this module:

QUESTION NAME	TOPIC AREA	DIFFICULTY
M10_Q01_INTEGRATED_ASSESSMENT	integrated assessment	foundational
M10_Q02_PRIORITY_SETTING	priority setting	foundational
M10_Q03_CAPSTONE_DOCUMENTATION	capstone documentation	foundational
M10_Q04_TEAM_COMMUNICATION	team communication	foundational
M10_Q05_PATIENT_ADVOCACY	patient advocacy	foundational
M10_Q06_TRANSITION_PLANNING	transition planning	applied
M10_Q07_PAYER_COMMUNICATION	payer communication	applied
M10_Q08_QUALITY_ESCALATION	quality escalation	applied
M10_Q09_HIGH_RISK_MANAGEMENT	high-risk management	applied
M10_Q10_FINAL_ASSESSMENT	final assessment	applied
M10_Q11_CERTIFICATE_RELEASE	certificate release	applied
M10_Q12_BRN_METADATA	BRN metadata	advanced
M10_Q13_CONTACT_HOUR_TOTAL	contact-hour total	advanced
M10_Q14_MOODLE_EVIDENCE	Moodle evidence	advanced
M10_Q15_APPROVAL_GUARDRAIL	approval guardrail	advanced

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Feedback/Attestation: Learner Reflection

Prompt: Describe one decision from this module that changed how you would coordinate care for a patient. Identify the patient-care risk, the RN case management action, and the documentation or escalation step that supports safe follow-up.

Attestation statement: I attest that I completed the required module activities myself, reviewed the learner content, participated in the active interaction, completed the documentation/application exercise, and answered the knowledge check questions.

Completion Gate: Required completion

Completion rule: Mark the module complete only after the learner views the overview and lesson pages, completes the scenario activity, submits the assignment, passes the knowledge check quiz, and submits the reflection/attestation.

Evidence Generated By Moodle

- Page completion logs.
- Lesson/H5P or interaction completion report.
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- Quiz attempt, score, item responses, and pass status.
- Reflection/attestation timestamp.
- Completion gate status.

Instructor/Admin Notes

Review documentation exercises for RN scope, patient-care relevance, factual language, and escalation accuracy. Do not insert provider approval language or certificate release settings until BRN CEP approval metadata is available. If instructor-specific feedback is provided, retain it with course records according to provider policy.

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